Statement of Deficiency Report

Department of Health
P.O. Box 47874, Olympia, WA 98504-7874
TEL: 360-236-4732

Comprehensive Healthcare – Two Rivers Landing
504 South 3rd Avenue
Yakima, WA 98902-3543
Agency Name and Address

Timothy Shields
Administrator

Investigation	Thursday, September 3, 2020	JAMC03
Inspection Type	Investigation Onsite Date	Investigator Number
2019-17430	RTF.FS.60068796	Mental Health
Case Number	License Number	BHA/RTF Agency Services Type

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the investigation.

Deficiency Number and Rule Reference	Observation Findings	Plan of Correction
WAC 246-337-065(5)(a) Safety and Security. The licensee must protect resident safety and security by developing written policies and procedures that are consistent with the requirements of this chapter and address: (5) reporting to the department and other appropriate agencies, by the end of the next business day of the incident occurring, serious or undesirable outcomes that occur in the facility including: (a) Allegations of abuse.	Based on record review, and policy and procedure review, the facility failed to report an allegation of abuse to the department by the end of the next business day of the incident occurring. Failure to report an allegation of abuse to the department by the end of the next business day of the incident occurring may result in patient harm. Findings included: 1. Investigator A requested documentation related to agency administration policy and procedures, which includes documentation on how the Facility reports	

	incidents of abuse. The Facility provided several policy and procedure documents, including "Authority of Board of Directors and CEO", "Delegation of Authority", and "Quality Improvement Program". Review of these policies and procedures showed that none of these polices included documentation related to the reporting of critical incidents. 2. Review of Department of Health (DOH) records showed that there was no report of the alleged sexual assault of Patient #1 to the Department of Health by the facility. 3. Review of an email, dated 09/08/20, showed that Staff A was unsure if the alleged assault of Patient #1 was reported to the Department of Health. Staff A wrote, " I am not sure if it was reported to DOH. Our safety officer at Comprehensive makes that determination ... I know we reported it to CLIP [Health Care Authority – Children's Long Term Inpatient Program] though because I did that one and they were very involved in the process".	
WAC 246-337-075(2)(a) Resident Rights. The licensee must establish a process to ensure resident rights are protected in compliance with chapter 7312 RCW, this chapter, and other applicable laws, and are based on the service types provided in the RTF. This process must address how the RTF will: (2) Treat each resident in a manner that respects individual identity, human dignity, and fosters constructive self-esteem. Each resident has the right to: (a) Be	Based on record review, policy and procedure review, and interview, the facility failed to ensure the resident was free of abuse. Failure to ensure the resident is free of abuse may result in patient harm. Findings included: 1. Review of police report, titled "Supplemental Narrative", dated 10/23/19, showed that Patient #1	

free of abuse, including being deprived of food, clothes, or other basic necessities.	reported sexual assault and harassment to the police. "[Patient #1] confirmed that there have been several incidents and I asked her to tell me about the first time that something happened. [Patient #1] told me [Police Detective] they were in a facility van coming back from an outing...[Patient #2] reached through a 'crease' in the van and he grabbed the left side of her body, under her shirt...[Patient #1] told me [Patient #2] grabbed her left breast when he reached through her shirt'." "I [Police Officer] told [Patient #1] I heard about something happening at the YMCA pool... [Patient#1] told me that she was swimming back and forth and [Patient #2] grabbed her crotch...[Patient #2] placed his hand down the back of her pants and then got out of the pool...". The police report showed that multiple other verbal incidents allegedly occurred, "[Patient #1] told me that the rest of the incidents were mostly verbal, although [Patient #2] played 'footsie' under the table." 2. During an interview on 8/26/20 at 12:15 PM ,Staff B , , Licensed Clinical Social Worker and Medical Provider, stated that he was involved in the ongoing treatment planning for Patient #1 and Patient #2. Staff B stated that he was aware of the ongoing issues between Patient #1 and Patient #2, and "it was appropriate for these two clients to go on outings, including going to the pool." Staff B stated that he felt that Facility Staff provided an appropriate level of patient supervision while on outings. 3. During an interview on 09/02/20 at 3:30 PM ,Staff D, Case Manager, stated that she was present at the pool outing where the alleged sexual assault occurred. Staff D stated, "The pool was new, it was our first time going there. When we got in there, we	

	noticed there were water tubes, a lazy river, there were all sorts of things going on. Staff didn't know where we were allowed to stand to monitor the clients. It was so new to us, it was like 3 to 5 minutes before she [Patient #1] was out of sight in the lazy river." When this interviewer asked if Staff D was able to maintain line of sight of all patients in that pool environment, she stated that she was not. Staff D stated that she was not aware of an alleged incident in the van involving Patient #1 and Patient #2. Staff D stated that there are always two staff that ride in the van with patients. One staff will drive, the other staff will observe the patients. The staff responsible for monitoring patients sometimes will ride in the passenger seat, and sometimes will ride in the back of the van with the patients. 4. Review of a Shift Note, dated 11/16/19, showed that staff D completed a report of the alleged sexual assault that occurred at the pool. "..while there staff attempted to keep a line of sight on [Patient #2], as he had a history of previous allegations made against him by this client [Patient #1]... the way the pool was set up was so that the staff could see the beginning of the lazy river from the seating area [where facility staff were sitting]. However, there was a large play structure that partially blocked the staff's view from the seating area... [Patient#1] stated that it happened on the lazy river... that the male client had touched her on her crotch and on her chest..." 5. Review of policy and procedure titled "Out of Facility Activities for Two Rivers Landing Clients", revised 9/26/19, showed that facility did not have guidance on the direct supervision of clients while on	

	outings. There was no documentation requiring staff to provide line of sight supervision while on outings. 6. Review of policy and procedure titled "Transportation of Clients utilizing Comprehensive Healthcare Services", revised1/10/20, showed that facility did not have guidance of direct supervision of patients while transporting patients in the facility van. There was no documentation requiring staff to provide line of sight supervision of patients while transporting patients. 7. Review of document titled "CLIP Meeting Agenda: Protocol for client outings with staff", undated, showed that a meeting agenda included discussion of staffing ratio requirements on outings. "Staff ratios: Always at least two staff present for up to 7 kids... increase staff ratios for day trips as needed." The document showed that the agenda included discussion of supervision requirements on outings, "Staff should always be able to have clear visual on client for outings...when possible, a staff should ride in the back to supervise clients while in the van...staff should accompany clients to all locations on an outing – if a client walks away from the group a staff member needs to follow or redirect client back to group."	

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	reports incidents of abuse. The Facility provided several policy and procedure documents, including "Authority of Board of Directors and CEO", "Delegation of Authority", and "Quality Improvement Program". Review of these policies and procedures showed that none of these polices included documentation related to the reporting of critical incidents. 2. Review of Department of Health (DOH) records showed that there was no report of the alleged sexual assault of Patient #1 to the Department of Health by the facility. 3. Review of an email, dated 09/08/20, showed that Staff A was unsure if the alleged assault of Patient #1 was reported to the Department of Health. Staff A wrote, " I am not sure if it was reported to DOH. Our safety officer at Comprehensive makes that determination ... I know we reported it to CLIP [Health Care Authority – Children's Long Term Inpatient Program] though because I did that one and they were very involved in the process".	
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	outing...[Patient #2] reached through a 'crease' in the van and he grabbed the left side of her body, under her shirt...[Patient #1] told me [Patient #2] grabbed her left breast when he reached through her shirt'." "I [Police Officer] told [Patient #1] I heard about something happening at the YMCA pool... [Patient#1] told me that she was swimming back and forth and [Patient #2] grabbed her crotch...[Patient #2] placed his hand down the back of her pants and then got out of the pool...". The police report showed that multiple other verbal incidents allegedly occurred, "[Patient #1] told me that the rest of the incidents were mostly verbal, although [Patient #2] played 'footsie' under the table." 2. During an interview on 8/26/20 at 12:15 PM ,Staff B , Licensed Clinical Social Worker and Medical Provider, stated that he was involved in the ongoing treatment planning for Patient #1 and Patient #2. Staff B stated that he was aware of the ongoing issues between Patient #1 and Patient #2, and "it was appropriate for these two clients to go on outings, including going to the pool." Staff B stated that he felt that Facility Staff provided an appropriate level of patient supervision while on outings. 3. During an interview on 09/02/20 at 3:30 PM ,Staff D, Case Manager, stated that she was present at the pool outing where the alleged sexual assault occurred. Staff D stated, "The pool was new, it was our first time going there. When we got in there, we noticed there were water tubes, a lazy river, there were all sorts of things going on. Staff didn't know where we were allowed to stand to monitor the clients. It was so new to us, it was like 3 to 5 minutes before she [Patient #1] was out of sight in the lazy river." When this interviewer	

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COMPREHENSIVE HEALTHCARE
POLICIES AND PROCEDURES

Title: High Risk & Sentinel Event Investigation and Reporting	**Policy #: PI-004**
Creation Date: June 30, 1999	**Revision Date(s):** *5/28/02; 5/10/04; 8/17/05; 4/19/06; 3/27/08; 5/14/2010; 5/10/12; 10/23/12; 10/09/13; 1/25/2016; 11/07/17; 8/15/18; 10/9/19; 10/7/20*
Chapter: Process Improvement **Folder:** Quality Improvement	**Affected Programs:** All
References: PI.01.01.01; PI.02.01.01; PI.03.01.01; NPSG.15.01.01; EC.02.01.01; LD.04.04.05; WAC 246-341-0420; WAC 246-341-0200; WAC 246-341-1000; WAC 246-377-065; WAC 388-78A-2640; RCW 74.34.020; MCO & ASO Contracts	**Attachments:** Risk Management Levels of Review; High Risk Investigation Report Form; Risk Review Template (Levels 2-4), Incident Report policy (LD-019)

Policy:
It is the policy of Comprehensive Healthcare to promptly respond to potential high risk or sentinel events in order to effectively manage the impacts of the event on the individuals and family members involved, learn from the incident (what worked well, staff actions or interventions that were effective in managing the situation or potentially interfered with the incident resolution, etc.), discover potentially unsafe environments or conditions, put into place corrective actions that further improve client care and service delivery, and reduce or eliminate potential future risks.

Requirements:
Six levels of risk reviews have been established and will be conducted based on the severity of the incident and the potential level of risk exposure (see Risk Review Levels attachment).

Anytime credible information is obtained regarding potential Sentinel or High Risk Events, Comprehensive's Risk Manager should be notified as soon as possible. After regular business hours, and on weekends, the supervising Director and VP (or their designee) is to be notified. If the supervising VP is not available the Chief Clinical Officer (CCO) is to be notified. The CEO will be notified by the Risk Manager, supervising VP, or CCO of all potential Sentinel or High Risk Events.

Events that represent a significant risk to the organization, but are not Sentinel Events will be identified for a formal High Risk Incident Review and root cause analysis (RCA). Sentinel Events will be voluntarily self-reported to the Joint Commission within five days of the event, with a complete RCA review and recommendations completed within 45 days of the event. Critical Incidents, as defined in state Behavioral Health WACs, will be reported to the appropriate state body overseeing that program or service.

External Notifications:
The Washington State Department of Health, Department of Social and Health Services, the Healthcare Authority, Managed Care Organizations (MCO), and regional Administrative Service Organizations (ASO). will be notified of critical or high risk events when appropriate after consultation with, and under the direction of, the CEO or designee (CCO, CAO, Risk Manager, or supervising Director or VP) according to the timelines and conditions listed below:

- The Risk Manager, or designee, will report and follow-up with the appropriate ASO or MCO Incident Manager, by telephone or secure email, within one business day any Critical Incidents as defined in WAC 246-341-0200 (see Definitions below) or as otherwise specified in the MCO or ASO contract.

- The Risk Manager, or designee, will report to the Department of Health within one business day the death, not on Comprehensive Healthcare's premises, of any individual enrolled in Opiate Treatment Program services within one day of receiving notice of the individual's death using the BHA Critical Incident Reporting webpage:
  https://fortress.wa.gov/doh/opinio/s?s=OTPDeathReporting

- The Risk Manager, supervising Director or VP, or designee of Residential or E&T Services will report to the Washington State Department of Health and other appropriate agencies by the end of the next business day safety and security incidents that involve serious or undesirable outcomes including:
  (a) Allegations of abuse;
  (b) Death;
  (c) Suicide;
  (d) Injuries resulting in an inpatient hospital stay; and
  (e) Disruption of services through internal or external emergency or disaster.

  Reports will be made using the BHA Critical Incident Reporting webpage:
  https://fortress.wa.gov/doh/opinio/s?s=ComplaintFormHPF

- The Risk Manager, supervising Director or VP, or designee of Residential Services licensed as an Assisted Living Facility will also notify DSHS/Residential Care Service within one working day of a residential program client's death, suicide, relocation to a hospital or other healthcare facility, or serious undesirable resident outcome.

  In addition, the supervising Director or designee in an Assisted Living Facility will consult with the resident's representative, the resident's physician, and other individual(s) designated by the resident as soon as possible whenever: there is a significant change in the resident's condition; the resident is relocated to a hospital or other health care facility; or the resident dies.

Definitions:

Critical Incident (as defined in WAC 246-341-0200)
Refers to any of the following events to be reported to the department, MCO or ASO, as applicable, within one business day of the critical incident:

(1) Any death, serious injury, or sexual assault that occurs at an agency that is licensed by the department;
(2) Alleged abuse or neglect of an individual receiving services, that is of a serious or emergency nature, by an employee, volunteer, licensee, contractor, or another individual receiving services;
(3) A natural disaster, such as an earthquake, volcanic eruption, tsunami, urban fire, flood, or outbreak of communicable disease that presents substantial threat to facility operation or client safety;
(4) A bomb threat;
(5) Theft or loss of data in any form regarding an individual receiving services, such as a missing or stolen computer, or a missing or stolen computer disc or flash drive;
(6) Suicide attempt at the facility;

(7) An error in program-administered medication at an outpatient facility that results in adverse effects for the individual and requires urgent medical intervention; and
(8) Any media event regarding an individual receiving services, or regarding a staff member or owner(s) of the agency.

High Risk Events
Events that expose Comprehensive to potential legal risk, occurred as a result of sub-standard care, resulted from unsafe environment or conditions, or had the potential to become a Sentinel Events (had the injury or loss of functioning been more severe, or if immediate steps had not been taken to prevent the worsening of the event). The incident review process also acknowledges that not all events are a result of staff actions or inaction. Some incidents are neither predictable nor preventable. The review process is designed to promote organizational learning and identify system changes that may reduce potential future risk.

Examples of high risk events include, but are not limited to:
- A death or loss of function following a discharge from treatment service against clinical staff's advice.
- Completed suicide of an individual engaged in outpatient treatment services.
- An event that resulted in temporary harm that could have resulted in severe temporary harm had minor changes in the conditions been present.
- Non-fatal suicide attempts that require medical attention.
- Report of an allegation of sexual assault or rape that does not meet the criteria for a Sentinel Event.

Sentinel Event
A sentinel event is a patient safety event (not primarily related to the natural course of an illness or underlying condition of an individual served) that reaches an individual served and results in any of the following:
- Death
- Permanent harm
- Severe temporary harm
    - Severe temporary harm is critical, potentially life-threatening harm lasting for a limited time with no permanent residual, but requires transfer to a higher level of care/monitoring for a prolonged period of time, transfer to a higher level of care for a life-threatening condition, or additional major surgery, procedure, or treatment to resolve the condition.

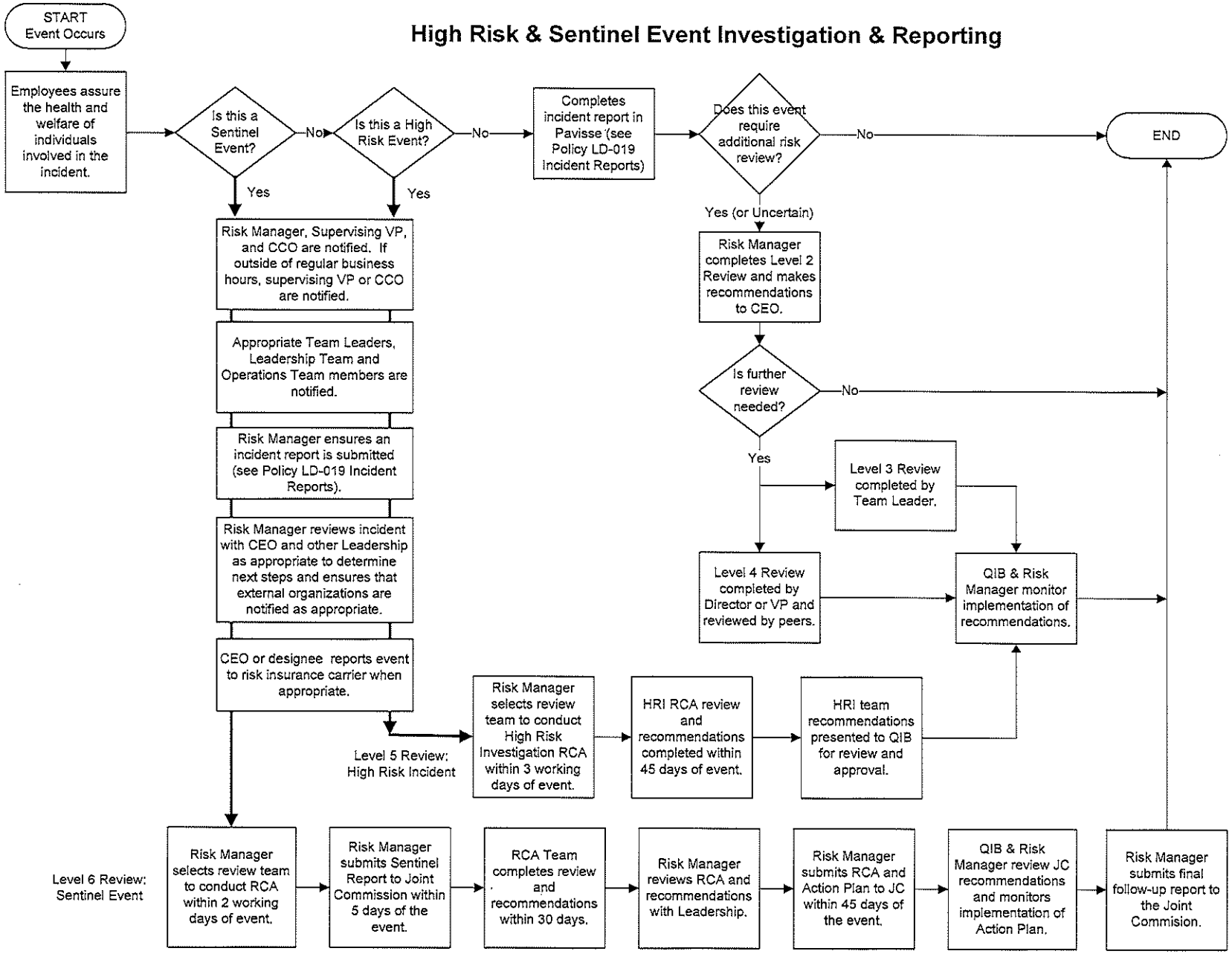
An event is also considered sentinel if it is one of the following:
- Suicide of any individual served receiving care, treatment, or services in a staffed around-the-clock care setting or within 72 hours of discharge, including from the organization's emergency department (ED)
- Abduction of any individual served receiving care, treatment, or services
- Any elopement (that is, unauthorized departure) of an individual served from a staffed around-the-clock care setting leading to the death, permanent harm or severe temporary harm of the individual served
- Rape, assault (leading to death, permanent harm, or severe temporary harm), or homicide of any individual served receiving care, treatment, or services while on site at the organization
    - Sexual abuse/assault (including rape) as a sentinel event is defined as nonconsensual sexual contact involving an individual served and another individual served, staff member, or other perpetrator while being treated or on the premises of the organization, including oral, vaginal, or anal penetration or fondling of the patient's sex organ(s) by another individual's hand, sex organ, or object. One or more of the following must be present to determine that it is a sentinel event:

  - Any staff-witnessed sexual contact as described above
  - Admission by the perpetrator that sexual contact, as described above, occurred on the premises
  - Sufficient clinical evidence obtained by the organization to support allegations of unconsented sexual contact
- Rape, assault (leading to death, permanent harm, or severe temporary harm), or homicide of a staff member, licensed independent practitioner, visitor, or vendor while on site at the organization.

Sentinel Event and High Risk Incident Review Team
Sentinel Event and High Risk Incident Reviews will be conducted under the direction of Comprehensive's Quality Improvement Board (QIB). It is the responsibility of the Risk Manager to convene a team who will be responsible for completion of the Root Cause Analysis (RCA) of the event. Members of this team will be selected on the basis of their functional knowledge related to the event. The Team shall include at least one member from Management. Other team members will be selected from staff across all levels of the organization. In no case should an individual staff member who has direct involvement in or relation to the event serve on the team. The team will have an identified leader, identified by the Risk Manager, who routinely provides status reports of the RCA on behalf of the team to the QIB. Staff members serving on the RCA team shall be protected from any disciplinary or adverse administrative action as a result of their direct participation in this process.

Root Cause Analysis
Root cause analysis (RCA) is a process for identifying the basic or causal factor(s) underlying variation in performance. Variation in performance can (and often does) produce unexpected and undesired adverse outcomes, including the occurrence or risk of a sentinel event. To be successful, the objective of an RCA must not be to assign individual blame. Rather, through the RCA review, a team works to understand a process or processes, the causes or potential causes of variation that can lead to error, and identify process changes that would make variation less likely to recur.

High Risk & Sentinel Event Investigation & Reporting



Procedures:

Responsible Party	Action
Employee (most familiar with event or designated to report the event)	☐ Assures the health, welfare and safety of individual(s) involved in an incident or potentially adverse event. o Medical assistance should be immediately sought for persons involved in the incident where physical injury to staff or clients has resulted. ☐ Notifies directly the supervising Director and VP of events requiring immediate attention such as potential Sentinel Events, High Risk Events, Critical Incidents, and any other significant incident. o Outside of regular business hours and on weekends, notify the supervising Director, VP or the VP's designee. If the VP is not available, notify the CCO. ☐ Notifies Risk Manager, or designee, if the event is potentially a Sentinel Event, a High Risk Event, a Critical Incident, or any other significant incident. ☐ Completes Incident Report in the Datix online Incident Reporting System (see policy LD-019 on Incident Reporting). o Add the Risk Manager as an Investigator in Datix to any event that may present a moderate to high risk to the organization.
Risk Manager	☐ Conducts Level 2 Reviews (see attachment) when requested or when risk level is uncertain. ☐ Notifies CEO of potential risk events. ☐ Ensures Incident Reports are completed and external organizations are notified as appropriate. ☐ Reviews Level 2 Reviews, High Incident Events and Sentinel Events with CEO or designee. ☐ Reports event to risk management insurance company and external agencies as directed by CEO within established timelines. ☐ Selects Sentinel Event and High Risk Investigation team members in consultation with Leadership. ☐ Supports RCA review teams and monitors completion of assigned tasks. ☐ Reviews Sentinel Event RCA results and recommendations with Leadership. ☐ Submits Sentinel Event RCA and Action Plan to Joint Commission within 45 days of the event. ☐ Monitors, in conjunction with the QIB, the implementation of RCA and other risk review recommendations. ☐ Submits final follow-up Sentinel Event report to Joint Commission confirming completion of the Action Plan.
Director of Residential and E&T Services	☐ Coordinates with supervising VP, Risk Manager, and Leadership on notifying external organizations consistent with the requirements outlined above.

Sentinel Event Investigation Team	☐ Meets as a committee and to conduct interviews at least weekly to complete review and recommendations. ☐ Utilizes RCA format to organize the investigation and make recommendations. ☐ Submits analysis and recommendations to Risk Manager within 30 days of the event for review by the QIB and Leadership.
High Risk Investigation Team	☐ Meets as a committee and to conduct interviews as needed to complete review and recommendations. ☐ Utilizes the High Risk Investigation form to organize the investigation and make recommendations. ☐ Submits analysis and recommendations to Risk Manager within 45 days of the event for review by the QIB and Leadership.
Quality Improvement Coordinator	☐ Supports Sentinel Event and HRI review teams with completing the RCA of assigned events. ☐ Reports to Risk Manager and QIB progress on reviews and final recommendations. ☐ Assists with monitoring implementation of review team corrective action recommendations.
Quality Improvement Board	☐ Evaluates Sentinel Event and HRI report recommendations. ☐ Recommends implementation of recommendations deemed necessary and appropriate. ☐ Monitors the implementation of action steps delegated to other employees or organizational committees.

COMPREHENSIVE HEALTHCARE
POLICIES AND PROCEDURES

Title: Out of Facility Activities for Two Rivers Landing Clients	**Policy #: PC-122**
Creation Date: November 8, 2016	**Revision Date(s):** *10/2/20*
Chapter: Provisions of Care, Treatment and Services **Folder:** Residential Services	**Affected Programs:** Two Rivers Landing
References:	**Attachments:**

Policy:
It is the policy of Comprehensive Healthcare to ensure that clients enrolled in CLIP treatment services at Two Rivers Landing are provided with the opportunity to be taken out of the facility as part of their treatment process.

Requirements:
All decisions to have clients leave the facility will require a recommendation by the treatment team that the client is able to manage the type of leave being requested. Clients will not be allowed to leave the facility if the level of supervision or clinical judgement indicates the client requires being on one-to-one supervision or line of sight at all times.

All transportation provided by Two Rivers Landing staff will follow the Transportation of Clients Utilizing Comprehensive Healthcare Services policy (PC-042).

Definitions:

School Leave: Transporting clients to a school environment for the purpose of completing education assessments and/or attending school during normal school hours.

Recreational Leave: Transporting clients out of the facility for any type of recreational activity under the supervisions of Two Rivers Landing staff.

Family/Social Support leave: Refers to a client leaving the facility with a family member or guardian as part of the treatment process to assist in the transition of a youth back into the community.

Procedures:
1. All decisions regarding clients leaving the facility will be made by the treatment team during multidisciplinary treatment team meetings.
2. The decision to allow clients to go on any type of leave will be documented in a multidisciplinary treatment team note of the client's EHR. Additionally, a provider will write an order indicating what type of leave the client may participate in. For school and recreation leave, this order will not be specified to a single leave and instead will be an indication that the client may participate in that type of leave.
3. All Family/Social Support leaves over four hours in length will require a provider's order written in the medical record, specified to that particular leave.
4. **Crisis Response School and Recreational Leave:**

a. If a client begins to demonstrate a loss of behavioral control during a leave
   i. Staff will immediately begin verbal de-escalation
   ii. Staff members will remove any other clients who are present
   iii. Staff will continue verbal de-escalation and crisis response.
   iv. Once a client has regained behavioral control staff will determine if it remains safe to continue the activity or if the client needs to be returned to the facility immediately
   v. Administrative and clinical supervision will be available on- call 24 hours a day, 7 days a week to assist in decision making process regarding the specific individual and the specific situation.
   vi. On all recreation leaves there will be a minimum of 2 staff members to assist in responding to the crisis
   vii. If the crisis occurs during business hours the supervisor will determine if additional staff are needed at the scene and will respond accordingly.
   viii. If after hours or on the weekend, in situations where an additional staff member is required the "rapid responder" for Comprehensive will be called to assist in the situation.
   ix. On school leave there may be one staff accompanying the clients. In situations where a client begins to escalate the staff will immediately call Two Rivers Landing and an additional staff member will go to assist in the situation.
   x. In the event that a client cannot regain behavioral control law enforcement will be called.
   xi. The staff may request law enforcement to assist in de-escalation and transportation back to Two Rivers Landing.

5. **Crisis Response during Family/Community Support Leave**
   a. Crisis response plans will be developed for all clients that are specific to the client, family, support system, and community
   b. All clients going on family leave will have a completed safety plan that has been reviewed with the appropriate people in the community.

STATE OF WASHINGTON

DEPARTMENT OF HEALTH

October 14, 2020

Comprehensive Healthcare – Two Rivers Landing
504 South 3rd Avenue
Yakima, WA 98902-3543

Re: Case Number: 2019-17430
License Number: RTF.FS.60068796
Acceptable Plan of Correction
Date(s) of Investigation: 05/11/20 -09/0/20

Dear Administrator:

This letter is to inform you that after careful review of the Plan of Correction (POC) you submitted for the investigation recently conducted at your facility, the Department has determined that the POC is acceptable. You stated in your plan that you will implement corrective actions by the specified timeline. By this, the Department is accepting your Plan of Correction as your confirmation of compliance.

Based on the scope and severity of the deficiencies listed in your statement of deficiency report, the Department will not conduct an unannounced follow-up compliance visit to verify that all deficiencies have been corrected.

The Department reserves the right to pursue enforcement action for any repeat and/or uncorrected deficiencies based on applicable statute and rules.

Investigator: JAM03
Department of Health
HSQA/Office of Health Systems Oversight
PO Box 47874
Olympia, Washington 98504-7874